

TOPIDERM HYDROCORTISONE 0.5% W/W CREAM

NAME AND STRENGTH OF ACTIVE SUBSTANCE

Hydrocortisone 0.5% w/w

DESCRIPTION

An opaque white to off-white viscous cream with characteristic odour.

PHARMACODYNAMIC

Mechanism of action

On topical application, corticosteroids produce anti-inflammatory, antipruritic and vasoconstrictor actions. The activity of the drugs is thought to result at least in part from binding with a steroid receptor. Corticosteroids decrease inflammation by stabilising leukocyte lysosomal membranes, preventing release of destructive acid hydrolyses from leukocytes; inhibiting macrophage accumulation in inflamed areas; reducing leukocyte adhesion to capillary endothelium; reducing capillary wall permeability and oedema formation; decreasing complement components; antagonising histamine activity and release of kinin from substrates; reducing fibroblast proliferation, collagen deposition, and subsequent scar tissue formation; and possibly by other mechanisms as yet unknown.

PHARMACOKINETICS

The rate and extent of hydrocortisone absorption through the skin varies among individual patients. Following topical application of a corticosteroid to most areas of normal skin, only minimal amounts of the lipophilic drug partitions into the predominantly aqueous dermo-epidermal layer (viable epidermis + dermis) and subsequently the systemic circulation.

Absorption is, however, markedly increased when the skin has lost its keratin layer or the rate limiting properties of the stratum corneum. Physical disruption of the stratum corneum, inflammation and/or disease of the epidermal barrier (eg. psoriasis, eczema) may result in increased absorption. Hydrocortisone is absorbed to a greater degree from the skin behind and around the ear region, scrotum, axilla, eyelid, face and scalp than forearm, knees, elbow, palm and sole. Prolonged absorption persists even after the area of application has been washed, possibly because the drug is retained in the stratum corneum and/or the dermo-epidermal layer.

Hydrocortisone is metabolised in the liver and most tissues to biologically inactive compounds, mainly glucuronides and sulphates. These are excreted in urine, mainly conjugated as glucuronides, together with a very small proportion of unchanged hydrocortisone. Some of the topical corticosteroids and their metabolites are also excreted in the bile.

Children are at a greater risk of systemic absorption of topical steroids due to higher permeation properties of the skin and increased surface area to body mass ratio.

INDICATION

For the temporary relief of symptoms associated with acute and chronic corticosteroid responsive conditions, including minor skin irritations, itching and rashes due to eczema, dermatitis, contact dermatitis (such as rashes due to cosmetics and jewellery), psoriasis, anogenital pruritis and sunburn.

RECOMMENDED DOSAGE

Adults and Children 2 years of age and older:

Apply to affected area not more than 3-4 times daily.

Children under 2 years of age: Do not use, consult a doctor.

ROUTE OF ADMINISTRATION

Topical

CONTRAINDICATIONS

- Tuberculous and fungal conditions of the skin, acute Herpes simplex, vaccinia, varicella and all viral infections.
- Hypersensitivity to any component of the cream .
- Not to be used in the eye.
- Topical steroids should not be used in patients with markedly impaired circulation since skin ulceration has occurred in these patients following the use of corticosteroids.
- Topical steroid should be used with caution and occlusive dressings should not be used in patients with primary skin infections.

WARNINGS AND PRECAUTIONS

FOR EXTERNAL USE ONLY. AVOID CONTACT WITH EYES.

If irritation develops, Topiderm cream should be discontinued and alternative therapy instituted.

Prolonged and heavy doses of hydrocortisone may have an immunosuppressant effect and thus decrease resistance to infection as well as mask signs of it. If infection of the skin is present suitable antifungal or antibacterial agents should be used first. Any occlusive dressings should be discontinued. If the infection does not respond promptly to therapy, corticosteroid therapy should be discontinued until the infection has been controlled.

Where very large areas are treated for long periods (e.g. atopic dermatitis), the possibility of systemic absorption exists, particularly if an occlusive dressing is applied. Prolonged use of large quantities of topical corticosteroids may also result in atrophic striae or acne eruptions.

Due to the possible systemic absorption of topical steroids, there may be a need for periodic evaluation of hypothalamo-pituitary-adrenal (HPA)-axis suppression by using the urinary free cortisol test or the corticotrophin stimulation test. If the HPA-axis suppression is evident, withdrawal should be attempted and the frequency of application reduced.

Manifestations of adrenal suppression in children include retardation of linear growth, delayed weight gain; low plasma cortisol concentrations and lack of response to corticotrophin stimulation (see PHARMACOLOGY - Pharmacokinetics). Manifestations of intracranial hypertension include bulging fontanelles, headache, and bilateral papilloedema. Topical corticosteroid therapy in children should be limited to the minimum amount necessary for therapeutic efficacy; chronic topical corticosteroid therapy may interfere with growth and development. Parents should be advised not to use tight-fitting diapers or plastic pants on a child being treated in the diaper area, since such garments may constitute occlusive dressings.

Children have a greater surface to mass ratio than adults, and so may be at greater risk of adverse events from increased dosages.

Any corticosteroid therapy tends to elevate blood glucose levels in diabetic patients, and this should be monitored during treatment.

Corticosteroids should be used cautiously in patients with non-specific ulcerative colitis, diverticulitis, colon abscess or other pyogenic infection, colon obstruction, or extensive fistulas and sinus tracts, fresh intestinal anastomoses, active or latent peptic ulcers, renal insufficiency, hypertension, osteoporosis and myasthenia gravis.

Topical corticosteroids should be used with caution in the management of psoriasis, as exacerbation of the disease or pustular psoriasis may occur during or on withdrawal of topical corticosteroid therapy.

Topical corticosteroids should also be used with caution in patients with impaired T cell function or in those patients receiving other immunosuppressive therapy. The immunosuppressive effects of corticosteroids may be associated with impairment of the normal function of T cells and macrophages. The result of this impairment may be the activation of latent infection or exacerbation of intercurrent infections, including those caused by Candida, Mycobacterium, Toxoplasma, Strongyloides, Pneumocystis, Cryptococcus, Nocardia and Amoeba.

Patients on long term therapy, if there is a risk of immunosuppression, should not be given any live attenuated vaccines.

INTERACTIONS WITH OTHER MEDICAMENTS

There are many potential drug interactions with hydrocortisone, However most are quite unlikely with topical therapy, occurring mainly with prolonged or over-use.

Carbamazepine, phenytoin and rifampicin all induce hepatic enzymes and thus lead to increased metabolism of hydrocortisone.

Concomitant use of diuretics, which also deplete potassium ion concentration in the blood, may cause hypokalaemia.

All corticosteroids antagonise the effects of neuromuscular blocking agents, such as vecuronium. Oral contraceptives have been shown to prolong the half-life of hydrocortisone and thus potentiate its anti-inflammatory effects.

USAGE IN PREGNANCY

Use in Pregnancy – Category A:

Appropriate studies in humans have not been done. In vivo studies using pregnant animals have shown that application of large amounts of topical corticosteroids, especially the more potent ones, over prolonged periods may cause foetal abnormalities. Therefore, topical corticosteroids should be used in pregnancy only when the potential benefits justify the possible risks to the foetus. The drugs should not be used on extensive areas, in large amounts or for prolonged periods in pregnant women.

USAGE IN LACTATION

It is not known whether topical corticosteroids are distributed into milk; however, systemic corticosteroids are distributed into milk. Topical corticosteroids should be used with caution in nursing mothers.

ADVERSE EFFECTS

An itching or burning sensation may be experienced when using this medication.

Topical corticosteroids may cause adverse dermatological effects. Adverse dermatological effects are most likely to occur in intertriginous and facial areas. Local adverse corticosteroid effects occur most frequently with occlusive dressings, especially with prolonged therapy, and may require discontinuation of the dressings. Atrophy of the epidermis, subcutaneous tissue and dermal collagen; drying and cracking or tightening of the skin may occur. Epidermal thinning, telangiectasia, increased fragility of cutaneous blood vessels, pupura and atrophic striae are also reported.

Other adverse dermatologic effects of topical corticosteroids include acneform eruption, vesiculation, irritation, pruritis, hypertrichosis, rosacea-like eruptions on the face, erythema, hyperthesia, perioral dermatitis, burning or stinging sensation, folliculitis, and hypopigmentation.

Adverse dermatological effects usually improve when the drug is discontinued but may persist for long periods; atrophic striae may be permanent. In addition to the other adverse dermatological effects of topical corticosteroid therapy, maceration of the skin and miliaria may occur, especially when occlusive dressings are used. Topically applied steroids are generally nonsensitising, but allergic contact dermatitis may occur rarely.

Topical corticosteroids should be used with caution in the management of psoriasis (see PRECAUTIONS).

Topical corticosteroids should also be used with caution in patients with impaired T cell function or in those patients receiving other immunosuppressive therapy. The result of this impairment may be the activation of latent infection or exacerbation of intercurrent infections (see PRECAUTIONS).

Any cardiovascular adverse events are unlikely with topical hydrocortisone therapy, however, prolonged prolific use may result in a transient hypertension as a result of fluid retention.

Long term corticosteroid use has resulted in Benign Intracranial Hypertension, with most reports occurring in children.

Corticosteroids have documented effects on serum lipids, including increased total cholesterol, increased low density lipoproteins and increased triglyceride levels.

Topical and systemic corticosteroid therapy has been implicated in posterior subcapsular cataract formation, elevated intraocular pressure, optic nerve damage, papilloedema and blurred vision (see PRECAUTIONS).

Cataracts, although primarily reported with systemic corticosteroid use, have been reported with use of topical preparations.

Hydrocortisone has least potential for causing glaucoma, but reports have demonstrated this complication in patients using topical preparations on the face.

OVERDOSE AND TREATMENT

Acute ingestion or accidental poisoning, even in massive doses, is rarely a clinical problem. Treatment should be symptomatic and supportive. Excessive chronic exposure results in adverse systemic effects. In such cases the use of topical corticosteroid should be discontinued, with the consideration to tapering the dose. Emesis or activated charcoal is not usually indicated unless multiple ingestion is suspected. Support the patient as necessary and treat symptomatically.

STORAGE CONDITION

Keep containers well closed. Protect from strong light. Store below 30 °C.

PACKAGING AVAILABLE

Collapsible aluminium tube of 30g with HDPE cap.

MANUFACTURER:

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